AVERY HOME HEALTH LLC

OASIS

5660 W Flamingo RdSTE A
Las Vegas NV 89103
Phone: (702)416-0611 Fax: (702)899-4479
NPI: 1720712235

STAFF INFO
Name: Berberova, Emiliya RN
Patient INFO
Name: NEWMAN, JOEL B.
DOB: 1939-03-26

OASIS Assessment: OASIS E2 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number:
297328

(M0014) Branch State:
Branch State: **Nevada**

(M0016) Branch ID Number:

(M0018) Attending Physician (NPI):
1700840659

Physician:
STEINHAUER, MARK

Physician Address:
12 COMMERCE CENTER DR # D-44 HENDERSON NV 89014

☐ Not Applicable

(M0020) Patient ID number:
123982

(M0030) Start of Care Date:
04/04/2026

(M0032) Resumption of Care Date:
mm/dd/yyyy

☑ Not Applicable

(M0040) Patient Name:

First Name:	Middle Name:	Last Name:	Suffix:
JOEL	B	NEWMAN	

(M0066) Birth Date:
03/26/1939

(A0810) Sex:
◉ 1. Male ○ 2. Female

(M0050) Patient State of Residence:
State of Residence: **Nevada**

(M0060) Patient Zip Code:
89121

(M0063) Medicare Number:
9JJ0U36XG11

☐ Not Applicable

(M0064) Social Security Number:

☑ Not Applicable

(M0065) Medicaid Number:

☑ Not Applicable

(M0080) Discipline Completing Assessment:
◉ 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Berberova, Emiliya RN

(M0090) Date Assessment Completed:
04/04/2026

(M0100) Assessment Reason:
◉ 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply:

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language:

A. What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
◉ 0. No ○ 1.Yes ○ 9. Unable to determine

(B1300) Health Literacy:
How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?
◉ 0. Never ○ 1. Rarely ○ 2. Sometimes ○ 3. Often ○ 4. Always ○ 7. Patient declines to respond ○ 8. Patient unable to respond

(M0102) Date of Physician-ordered Start of Care (Resumption of Care):
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1255, Transportation, if date entered:
mm/dd/yyyy

☑ NA - No specific SOC/ROC date ordered by physician

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
04/03/2026

(M0150) Current Payment Sources for Home Care:
(Mark all that apply)

- ☐ 0. None; no charge for current services
- ☑ 1. Medicare (Traditional fee-for-service)
- ☐ 2. Medicare (HMO/managed care/Advantage plan)
- ☐ 3. Medicaid (Traditional fee-for-service)
- ☐ 6. Title programs (for example, Title III, V, or XX)
- ☐ 7. Other government (for example, TriCare, VA)
- ☐ 8. Private insurance
- ☐ 9. Private HMO/managed care

- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1255) Transportation (NACHC©):

In the past 12 months, has lack of reliable transportation kept you from medical appointments, meetings, work or from getting things needed for daily living?

○ 0. Yes ● 1. No ○ 7. Patient declines to respond ○ 8. Patient unable to respond

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(M1000) Inpatient Facilities that patient was discharged within the past 14 days?

(Mark all that apply)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [x] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [ ] NA - Patient was not discharged from an inpatient facility

(M1005) Inpatient Discharge Date (Most Recent):

04/03/2026 [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity (POC Element):

The patient is considered homebound because he is unable to leave the home safely or without considerable and taxing effort due to objective mobility, cognitive, and medical limitations documented during his recent hospitalization. Specifically, the record documents that he ambulates only 3 feet and requires staff assistance; he needs moderate assistance to get in and out of bed and into the bathroom, moderate assistance with upper-body dressing, maximal assistance with lower-body dressing and bathing, and total assistance with stair climbing (he reports 16 stairs to his living area). He also has notable cognitive impairment and was described as confused with episodes of delirium, making him a poor historian and increasing fall and safety risk with unassisted community ambulation. Medically, he experienced new-onset atrial fibrillation with a transient rapid ventricular rate (heart rate documented in the 160s on presentation and managed on telemetry), a recent acute kidney injury with a creatinine peak of 1.63, and ongoing cardiovascular and metabolic comorbidities (history of CABG, hypertension, type 2 diabetes, hyperlipidemia) that heighten the risk of decompensation with exertion. He is anticoagulated with apixaban (Eliquis), which increases the clinical risk associated with falls. Taken together, these findings show that leaving the home would require a considerable and taxing effort given his need for close supervision and hands-on assistance for transfers and ADLs, impaired cognition, recent unstable cardiac rhythm and renal injury, and lack of reliable in-home caregiver support while his wife is hospitalized. Allowable, limited absences are restricted to medically necessary appointments and brief supervised/community excursions as tolerated (for example, brief supervised walks or attended religious services), with the expectation that any outing will require accompaniment and direct assistance. These functional deficits, safety concerns, and recent medical instability meet Medicare's homebound criteria and justify the medical necessity for skilled home health services (skilled nursing, rehabilitative PT/OT, and speech therapy) to monitor cardiac/renal status, provide hands-on training and ADL assistance, reduce fall risk, and promote safe transition home.

Homebound Reason (POC Element):

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [x] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadriplegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Experiencing pain impairs/restricts mobility
- [ ] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [x] Leaving home is exhausting, extended rest periods to recover
- [x] Extended respiratory recovery after exertion
- [x] Requires human assist due to unsteady gait/poor balance
- [ ] Weakness and pain from recent surgery
- [ ] Visually impaired-requires constant attendance/assist
- [ ] Cognitively impaired, unable to safely leave home without assist
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [ ] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis (POC Element (§484.60 (2.v))):

( ) 1-Poor ( ) 2-Guarded (x) 3-Fair ( ) 4-Good ( ) 5-Excellent

Functional Limitations (POC Element (§484.60 (2.vi))):

( ) 1 - Amputation
( ) 2 - Bowel/Bladder (Incontinence)
( ) 3 - Contracture
(x) B- Other (Specify)
Dyspnea with moderate exertion
+Pain
( ) 4 - Hearing
( ) 5 - Paralysis
(x) 6 - Endurance
(x) 7 - Ambulation
( ) 8 - Speech
( ) 9 - Legally blind
( ) A - Dyspnea with minimal exertion

Emergency Preparedness (POC Element):

Obtained the patient's emergency contact numbers for the medical record
Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Discussed patient specific emergency planning options
List of recommended items to have prepared/ready and available in the event of an emergency
Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient, and Representative.
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted

Acuity Level:
Acuity Level: **Medium Risk or Medium Priority**

Emergency Triage:
Emergency Triage: **Category 3 - Visit could be postponed 24-48 hours without adverse effects**

Additional Emergency Preparedness Information:

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives (POC Element (§484.60 (2.xv))):

Patient has Advance Directives? (x) Yes ( ) No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [x] Power of Attorney — Copies to Follow
- [x] Living Will — Copies to Follow
- [ ] The patient has not executed any advance directives to health care.
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [ ] Full Code
- [x] The patient was provided with written and verbal information on advance directives.

Comments:
Power of Attorney Additional Info:

Has surrogate? ( ) Yes (x) No

Surrogate Name:

Surrogate Phone:

Health Screening:

Last Cholesterol Level: mm/dd/yyyy

Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ( ) Yes (x) No

Last Pap Smear: mm/dd/yyyy

Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy

Last Colonoscopy: mm/dd/yyyy

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control (POC Element (§484.60 (2.i))):

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control

appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

	Onset Date	Severity						
PRIMARY DIAGNOSIS G93.41 - Metabolic encephalopathy	04/04/2026	0	1	2	(3)	4	^	Onset / (Exacerbate)
OTHER DIAGNOSIS - 1 I48.91 - Unspecified atrial fibrillation	04/04/2026	0	1	2	(3)	4	^	Onset / (Exacerbate)
OTHER DIAGNOSIS - 2 E11.9 - Type 2 diabetes mellitus without complications	04/04/2026	0	1	(2)	3	4	^	(Onset) / Exacerbate
OTHER DIAGNOSIS - 3 I10. - Essential (primary) hypertension	04/04/2026	0	1	(2)	3	4	^	(Onset) / Exacerbate
OTHER DIAGNOSIS - 4 E78.5 - Hyperlipidemia, unspecified	04/04/2026	0	1	(2)	3	4	^	(Onset) / Exacerbate
OTHER DIAGNOSIS - 5 C18.9 - Malignant neoplasm of colon, unspecified	04/04/2026	0	1	(2)	3	4	^	(Onset) / Exacerbate
G47.33 - Obstructive sleep apnea (adult) (pediatric)	04/04/2026	0	1	(2)	3	4	^	(Onset) / Exacerbate
Z91.81 - History of falling	04/04/2026	0	1	2	3	4	(^)	(Onset) / Exacerbate
Z95.1 - Presence of aortocoronary bypass graft	04/04/2026	0	1	2	3	4	(^)	(Onset) / Exacerbate
Z79.01 - Long term (current) use of anticoagulants	04/04/2026	0	1	2	3	4	(^)	(Onset) / Exacerbate
Z79.84 - Long term (current) use of oral hypoglycemic drugs	04/04/2026	0	1	2	3	4	(^)	

(●) Onset ( ) Exacerbate

(M1028) Active Diagnoses:

Comorbidities and Co-existing Conditions *– Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.*

- [ ] **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- [x] **2. Diabetes Mellitus (DM)**
- [ ] **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up:

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
72

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
190

BMI:
25.77

Comments:

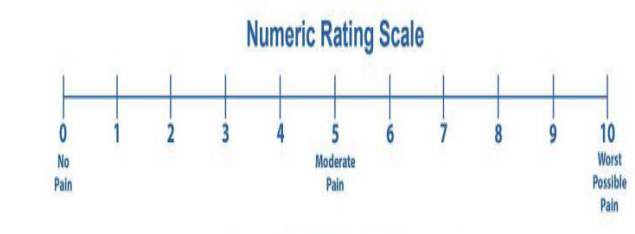
Vital Signs

Temperature: 97.4 Temporal
Respiratory (per minute): 16
Blood Pressure mm/Hg (R): /
Blood Sugar (mg/dl): 125 Non-Fasting
O2 Sat: 95
Comments:

Pulse (per minute): Apical = Radial = 60
Weight:
Blood Pressure mm/Hg (L): 120 / 64 sitting

O2 Sat in: Room Air

Pain Assessment



4

Collected Using: [ ] Face Scale [x] Numeric Rating Scale

Pain Location and Description:

1 **Location**
Right hip

Start Date
04/04/2026

Activity
With Activity×

What makes pain worse?
Activity×

Intensity
4×

Duration
hours

What makes pain better?
Med/Rest×

Does pain affect quality of life?
yes

Unable to Assess [ ]

Description
Dull Ache×

Frequency
Frequent×

Unable to Assess [ ]

Type of Intervention
Non Phamacologic×

How well it is working
Fairly Well×

Status
COC×

Comments:
Pain interferes with ADLs. Pain contributes to decreased mobility, limited endurance, and difficulty with prolonged ambulation and activity tolerance, increasing fall risk and the need for pacing of activities.

(J0510) Pain Effect on Sleep:

Ask patient: *"Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"*

- ( ) **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ( ) **1. Rarely or not at all**
- (●) **2. Occasionally**
- ( ) **3. Frequently**
- ( ) **4. Almost constantly**
- ( ) **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities:

Ask patient: *"Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"*

- (●) **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ( ) **1. Rarely or not at all**
- ( ) **2. Occasionally**
- ( ) **3. Frequently**
- ( ) **4. Almost constantly**
- ( ) **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities:

Ask patient: *"Over the past 5 days, how often you have limited your day to day activities (excluding rehabilitation therapy session) because of pain?"*
○ **1. Rarely or not at all** ○ **2. Occasionally** ● **3. Frequently** ○ **4. Almost Constantly** ○ **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

Medication (POC Element (§484.60 (2.x))):

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	04/04/2026	DULoxetine (Oral Pill)	30 mg 1 Cap Twice a day	By mouth	ANTIDEPRESSANTS,OTHER /	New
☐	04/04/2026	RYBELSUS	3 mg 1 Tab Once a day	By mouth	HYPOGLYCEMIC AGENTS,OTHER /	New
☐	04/04/2026	dilTIAZem (Oral Pill)	60 mg 1 Tab Three times a day	By mouth	CALCIUM CHANNEL BLOCKERS /	New
☐	04/04/2026	Metoprolol (Oral Pill)	25 mg 1 Tab Twice a day	By mouth	BETA BLOCKERS/RELATED /	New
☐	04/04/2026	ELIQUIS	5 mg 1 Tab Once a day	By mouth	ANTICOAGULANTS /	New
☐	04/04/2026	Atorvastatin (Oral Pill)	20 mg 1 Tab Once a day	By mouth	ANTILIPEMIC AGENTS /	New
☐	04/04/2026	Albuterol (Inhalant)	90 mcg/puff 2 Puffs As needed	Inhalant	BRONCHODILATORS,SYMPATHOMIMETIC,INHALATION / for shortness of breath/wheezing	New
☐	04/04/2026	ADVAIR	100-50 mcg/puff 2 Puffs Twice a day	Inhalant	ANTIASTHMA,OTHER /	New
☐	04/04/2026	VITAMIN D3	1,000 iu 1 Tab Once a day	By mouth	VITAMIN D,OTHER /	New

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Allergies (POC Element):

Name	Reaction	Start Date
		None known

☐ **Add Allergies**

(M2001) Drug Regimen Review:
Did a complete drug regimen review identify potential clinically significant medication issues?
○ **0. No – No issues found during review [Go to M2010, Patient/Caregiver High-Risk Drug Education]**
● **1. Yes – Issues found during review**
○ **Not assessed/no information**
○ **9. NA – Patient is not taking any medications [Go to O0110, Special Treatments, Procedures, and Programs]**

(M2003) Medication Follow-up:
Did the agency contact a physician (or physician designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?
○ **0. No** ● **1. Yes** ○ **Not assessed/no information**

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking 1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication Noted 2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	■
E. Anticoagulant	☑	☑
F. Antibiotic	☐	■
H. Opioid	☐	■
I. Antiplatelet	☐	■
J. Hypoglycemic (including insulin)	☑	☑
Z. None of the Above	☐	

(M2010) Patient/Caregiver High-Risk Drug Education:
Has the patient/caregiver received instruction on special precautions for all high risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?
○ **0. No**
● **1. Yes**
○ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

(M2020) Management of Oral Medications:

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. ***(NOTE: This refers to ability, not compliance or willingness.)***

- ◯ **0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ◯ **1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ◯ **2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ● **3. Unable to take medication unless administered by another person**
- ◯ **NA - No oral medications prescribed**

(M2030) Management of Injectable Medications:

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ◯ **0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ◯ **1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ◯ **2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ◯ **3. Unable to take injectable medication unless administered by another person**
- ● **NA - No injectable medications prescribed.**

(O0110) Special Treatments, Procedures, and Programs:

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations:

Pneumonia:	**Flu:**	**Tetanus:**	**TB:**
☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy
TB Exposure:	**Hepatitis B:**	**COVID-19:**	**Additional Immunization:**
☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy

Comments:

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Neuromuscular:

☐ No Problems Identified

Level of Consciousness:

☑ Awake ☑ Alert ☐ Lethargic ☐ Minimally Responsive ☐ Unresponsive ☐ Coma

Impairment:

☐ Speech ☑ Hearing ☑ Visual ☐ Tremors ☐ Numbness ☐ Tingling ☐ Vertigo ☐ Ataxia ☐ Decreased Sensitivity

Speech Impairment (Specify):

Psychosocial (POC Element (§484.60 (2.ii))):

☐ Document as Clinical Narrative

Educational level

Primary Language

English

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language

◯ Needs interpreter

Learning

◯ Mental
● Physical

◯ Psychosocial
◯ Functional

Emotions Reported:

◯ Angry
◯ Discouraged
◯ Withdrawn

◯ Flat affect
◯ Difficulty coping
◯ Disorganized

● Depressed ● Recent ◯ Long Term

Treatment:

Duloxetine 30 mg, 1 capsule by mouth twice daily

Evidence of abuse/neglect/exploitation:

◯ Potential
◯ Verbal/Emotional
◯ Physical

◯ Actual
◯ Financial
● No Evidence of Abuse/Neglect/Exploitation

Unable to cope with altered health status, patient:

◯ Lacks motivation ◯ Has unrealistic expectations ◯ Denies problems ● Unable to recognize problems

Sleep/Rest

● Adequate ◯ Inadequate

No. of hours slept per night: 7

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☐Yes ☑No ☐NA

Did the patient have a job before admission? ◯Yes ◉No

If yes, do they want to drive post-discharge? ◯Yes ◯No

If yes, do they want to return to work post-discharge? ◯Yes ◯No

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ● **Two** ◯ **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year

◯ 0. Missed by > 5 years or no answer
◯ 1. Missed by 2-5 years
◯ 2. Missed by 1 year
● 3. Correct
◯ Not assessed/no information

Ask patient: **"What month are we in right now?"**

B) Able to report correct month

◯ 0. Missed by > 1 month or no answer
◯ 1. Missed by 6 days to 1 month
● 2. Accurate within 5 days
◯ Not assessed/no information

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week

◯ 0. Incorrect or no answer
● 1. Correct
◯ Not assessed/no information

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"

● 0. No – could not recall
◯ 1. Yes, after cueing ("something to wear")

B) Able to recall "blue"

◯ 0. No – could not recall
◯ 1. Yes, after cueing ("a color")

C) Able to recall "bed"

◯ 0. No – could not recall
◯ 1. Yes, after cueing ("a piece of furniture")

◯ 2. Yes, no cue required ◯ Not assessed/no information	✓ 2. Yes, no cue required ◯ Not assessed/no information	✓ 2. Yes, no cue required ◯ Not assessed/no information

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** — **12**

(C1310) Signs and Symptoms of Delirium (from CAM©):

A. Acute Onset of Mental Status Change:

Is there evidence of an acute change in mental status from the patient's baseline?

✓ 0. No ◯ 1. Yes

B. Inattention:

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

◯ 0. Behavior not present
◯ 1. Behavior continuously present, does not fluctuate
✓ 2. Behavior present, fluctuates (comes and goes, changes in severity)
◯ Not assessed/no information

C. Disorganized thinking:

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

◯ 0. Behavior not present
◯ 1. Behavior continuously present, does not fluctuate
✓ 2. Behavior present, fluctuates (comes and goes, changes in severity)
◯ Not assessed/no information

D. Altered level of consciousness:

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- ***vigilant*** *– startled easily to any sound or touch* ☐ *lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch*
- ***stuporous*** *– very difficult to arouse and keep aroused for the interview*
- ***comatose*** *– could not be arouse*

✓ 0. Behavior not present
◯ 1. Behavior continuously present, does not fluctuate
◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
◯ Not assessed/no information

(D0150) Patient Mood Interview (PHQ-2 to 9):

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	Yes ×	2-6 days (several days) ×
B. Feeling down, depressed, or hopeless	Yes ×	2-6 days (several days) ×
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		2

(D0700) Social Isolation:

How often do you feel lonely or isolated from those around you?

◯ 0. Never ◯ 1. Rarely ✓ 2. Sometimes ◯ 3. Often ◯ 4. Always ◯ 7. Patient declines to respond ◯ 8. Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed):

(Mark all that apply)

☐ 1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
☑ 2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
☐ 3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
☐ 4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
☐ 5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
☐ 6. Delusional, hallucinatory, or paranoid behavior
☐ 7. None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

◯ 0. Never ◯ 1. Less than once a month ◯ 2. Once a month ✓ 3. Several times each month ◯ 4. Several times a week ◯ 5. At least daily

(M1700) Cognitive Functioning:

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

◯ 0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
✓ 1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.
◯ 2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility
◯ 3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
◯ 4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- ○ 0. Never
- ● 1. In new or complex situations only
- ○ 2. On awakening or at night only
- ○ 3. During the day and evening, but not constantly
- ○ 4. Constantly
- ○ 5. Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- ○ 0. None of the time
- ● 1. Less often than daily
- ○ 2. Daily, but not constantly
- ○ 3. All of the time
- ○ NA - Patient nonresponsive

Mental Status (POC Element (§484.60 (2.ii)):

- ● 1 - Oriented
- ○ 2 - Comatose
- ● 3 - Forgetful
- ○ 4 - Depressed
- ○ 5 - Disoriented
- ○ 6 - Lethargic
- ○ 7 - Agitated
- ● 8 - Alert
- ○ 9 - Confused
- ○ 10 - Dementia
- ○ 11 - Assaultive
- ○ 12 - Delirious
- ○ 13 - Wanders
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is alert, oriented, forgetful at times, able to communicate needs, able to follow instructions, cooperative, and verbally responsive. Patient experiences episodes of depression. Thought content was normal with no psychotic or suicidal thoughts. The patient's judgment was realistic with normal insight into their present condition.

Has there been a sudden/acute change in their mental status? ☐ Yes ☐ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

Cultural:

Is religion important to the patient? ● Yes ○ No

Patient's primary source of emotional support:
Family

Eyes and Ears

(B1000) Vision:

Ability to see in adequate light *(with glasses or other visual appliances)*

- ○ 0. Adequate – sees fine detail, such as regular print in newspapers/books
- ● 1. Impaired – sees large print, but not regular print in newspapers/books
- ○ 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- ○ 3. Highly impaired – object identification in question, but eyes appear to follow objects
- ○ 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision Assessment:

- ☐ No Problems Identified
- ☑ Glasses
- ☐ Contact Lenses: ☐ Right ☐ Left
- ☐ Glaucoma: ☐ Right ☐ Left
- ☐ Blurred Vision: ☐ Right ☐ Left
- ☐ Macular Degeneration: ☐ Right ☐ Left
- ☐ Blindness: ☐ Right ☐ Left
- ☐ Cataracts: ☐ Right ☐ Left
- ☐ Other (Specify):

Pupils:

- ☑ PERRLA
- ☐ Anisocoria: ☐ Right ☐ Left
- ☐ Other (Specify):

Comments:

(B0200) Hearing:

Ability to hear *(with hearing aid or hearing appliances if normally used)*

- ○ 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- ● 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- ○ 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- ○ 3. Highly impaired – absence of useful hearing

Hearing Assessment:

- ☐ No Problems Identified
- ☑ Right Hearing Impairment
- ☑ Left Hearing Impairment
- ☐ Deaf
- ☑ Hearing Aids: ☑ Right ☑ Left

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds:

- ☐ No Problems Identified
- ☑ Irregular: ☑ Known Hx ☐ New Finding
- ☐ Gallops: ☐ Known Hx ☐ New Finding
- ☐ Clicks: ☐ Known Hx ☐ New Finding
- ☐ Murmur: ☐ Known Hx ☐ New Finding
- ☐ FrictionRub: ☐ Known Hx ☐ New Finding

Comments:
Patient with atrial fibrillation

Edema:

- ☐ Rigth Upper Extremities:
- ☐ Left Upper Extremities:
- ☐ Rigth Lower Extremities:
- ☐ Left Lower Extremities:

Others:

Others:

Comments:
No edema was noted

Peripheral Pulses:

Radial: ☑ Strong ☐ Bounding ☐ Weak ☐ Thready ☐ Absent **Pedal:** ☑ Strong ☐ Bounding ☐ Weak ☐ Thready ☐ Absent

Fluid Imbalance and Systemic Distress:
☐ Cramps ☐ Claudication ☑ Fatigue ☐ Face ☐ Neck ☐ Abdomen

Capillary Refill:
☑ < 3 secs ☐ > 3 secs

Chest/Thorax

Chest Pain:
☑ Denies ☐ Anginal ☐ Postural ☐ Localized ☐ Substernal ☐ Sharp
☐ Dull ☐ Aching ☐ Radiating ☐ Viselike ☐ Stabbing ☐ Nausea
☐ Other (Specify):

Associated with:
☐ Shortness of Breath ☐ Activity ☐ Rest ☐ Diaphoresis ☐ Frequency/Duration:

(M1400) When is the patient dyspneic or noticeably Short of Breath?
- ○ 0. Patient is not short of breath
- ○ 1. When walking more than 20 feet, climbing stairs
- ● 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- ○ 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- ○ 4. At rest (during day or night)

Respiratory Status:

Respiratory Distress Indicators:
☑ Dyspnea: ☐ At Rest ☑ With Exertion/Activity ☐ Accessory Muscle Used ☐ Orthopnea ☐ Retraction ☐ Stridor
☐ O2 Liters per minute: ☐ O2 Saturation (%):

Frequency/Timing of Oxygen Use:
☐ PRN ☐ Continuous ☐ Only at bedtime ☐ Other (Specify):

Route/Delivery:
☐ Nasal Cannula ☐ Trach Cuff ☐ Mask (Specify): ☐ Other (Specify):

Lung Sounds:

Anterior Right:	Anterior Left:		
Anterior Right: **Clear**	Anterior Left: **Clear**		
Posterior Upper Right:	**Posterior Upper Left:**	**Posterior Lower Right:**	**Posterior Lower Left:**
Posterior Upper Right: **Clear**	Posterior Upper Left: **Clear**	Posterior Lower Right: **Clear**	Posterior Lower Left: **Clear**

Comments:
SN instructed the patient on energy-conservation techniques, such as taking frequent rests/stops when walking, which can help prevent shortness of breath.

Cough:

Cough Characteristics:
☑ None ☐ Dry ☐ Acute ☐ Chronic

Productivity of Cough:
☐ Non-Productive
☐ Productive:

Sputum Characteristics (if Productive):
☐ Brown ☐ Clear ☐ Cloudy ☐ Bloody ☐ Yellow ☐ White
☐ Hemoptysis: ☐ Amount (Specify): ☐ Frequency (Specify):

GASTROINTESTINAL & GENITOURINARY

Gastrointestinal (Mouth - Anus)

Mouth and Throat (Assess oral health and swallowing ability):
☐ Dentures: ☐ Upper ☐ Lower ☐ Partial
☐ Bleeding Gums/Gingivitis ☐ Toothache ☐ Ulcerations ☐ Lesion
☐ Dysphagia ☐ Hoarseness ☐ Sore Throat ☐ Masses/Tumors

Comments:

(M1870) Feeding or Eating:
*Current ability to feed self meals and snacks safely. Note: This refers only to the process of **eating**, **chewing**, and **swallowing**, not preparing the food to be eaten.*
- ○ 0. Able to independently feed self.
- ● 1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- ○ 2. Unable to feed self and must be assisted or supervised throughout the meal/snack.
- ○ 3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- ○ 4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- ○ 5. Unable to take in nutrients orally or by tube feeding.

Appetite (%):	Appetite Percentage of (Meals per day):	Snacks (Per day):
80	3	

Appetite:
☐ Increase ☐ Decreased ☑ Unchanged

Intake:
☑ Adequate ☐ Inadequate

Nutritional Abnormalities:
☐ Anoxeria ☐ Cachexia ☐ Decreased Appetite ☐ Constipation

(K0520) Nutritional Approaches:

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	☐
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	☐
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	☐

D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	☑
Z. None of the above	☐

#16 Nutritional Requirements (POC Element (§484.60 (2.ix))):

☐ NPO ☐ No Added Salt ☑ Low Salt ☑ Low Fat ☑ Low Cholesterol ☑ No Concentrated Sweets
☑ Controlled Carbohydrates ☐ Regular ☐ Fluid Restriction:
Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
○ Without reason, has lost more than 10 lbs in the last 3 months	15	○ Does not always have enough money to buy foods need	10
● Has an illness or condition that made pt change the type and/or amount of food eaten	10	○ Eats alone most of the time	5
○ Has open decubitus, ulcer, burn or wound	10		
○ Eats fewer than 2 meals a day	10	● Takes 3 or more prescribed or OTC medications a day	5
○ Has a tooth/mouth problem that makes it hard to eat	10	○ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
○ Has 3 or more drinks of beer, liquor or wine almost every day	10	○ Frequently has a diarrhea or constipation	5
		Total	15

● Good Nutritional Status (Score 0 - 25) ○ Moderate Nutritional Status (Score 25 - 54) ○ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:
SN educated the patient and PCG on the prescribed diet and the importance of proper nutrition and hydration.

Weight:
☑ No Change ☐ Weight Gain ☐ Weight Loss
Comments:

(M1620) Bowel Incontinence Frequency:
● 0. Very rarely or never has bowel incontinence
○ 1. Less than once weekly
○ 2. One to three times weekly
○ 3. Four to six times weekly
○ 4. On a daily basis
○ 5. More often than once daily
○ NA - Patient has ostomy for bowel elimination
○ UK - Unknown

(M1630) Ostomy for Bowel Elimination:
Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?
● 0. Patient does not have an ostomy for bowel elimination
○ 1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
○ 2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

Bowel Sounds (Abdominal Quadrants):

Right Lower:	Right Upper:	Left Lower:	Left Upper:	Last Bowel Movement:
Right Lower: Normoactive	Right Upper: Normoactive	Left Lower: Normoactive	Left Upper: Normoactive	04/03/2026

GI Symptoms:
☐ Nausea ☐ Vomiting ☐ Reflux/Indigestion ☐ Bowel Incontinence ☐ Diarrhea ☐ Constipation

Abdominal Assessment:
☐ Abdominal Soft/Non-tender ☑ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm):

Hemorrhoids:
☐ External Hemorrhoids ☐ Internal Hemorrhoids ☐ Laxative/Enema Use

Stool Appearance:
☐ Gray ☐ Tarry ☐ Brown/Tan ☐ Black ☐ Fresh Blood ☐ Other (Specify):

Ostomy/Stoma (if applicable):
☐ Moist ☐ Dry ☐ Pinkished-Red ☐ Purple ☐ Gray ☐ Black ☐ Odor

Peristomal Skin Characteristics:
☐ Clean ☑ Healthy ☐ Red ☐ Weeping ☐ Peeling ☐ Inflamed ☐ Other (Specify):
Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?
● 0. No ○ 1. Yes ○ NA - Patient on prophylactic treatment ○ UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:
○ Patient is incontinent
- ○ Day time
- ○ Night time
- ○ Day and Night
- ○ Timed-voiding defers incontinence
- ○ Occasional stress incontinence

○ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)
● No incontinence or catheter (includes anuria or ostomy for urinary drainage)

Genitourinary Assessment:
☐ No Problems Identified

Abnormalities:

- [x] Frequency
- [ ] Dysuria
- [x] Urgency
- [ ] Anuria
- [ ] Nocturia
- [ ] Oliguria
- [ ] Retention
- [ ] Hematuria
- [ ] Irrigation
- [ ] Incontinence
- [ ] Diapers Used
- [ ] Penile Discharge
- [ ] Vaginal Bleeding/Discharge
- [ ] Other (Specify):

Urine Color:

- [ ] Transparent
- [x] Pale Yellow
- [ ] Brownish Orange
- [ ] Pinkish Red
- [ ] Dark Yellow
- [ ] Blue or Green
- [ ] Foamy
- [ ] Transparent Yellow

Urine Consistency:

- [x] Clear
- [ ] Sediment
- [ ] Odorous
- [ ] Cloudy

Urinary Catheter:

- [ ] Yes
- [x] No

- [ ] External Catheter
- [ ] Foley
- [ ] Suprapubic
- [ ] Nephrostomy
- [ ] Last Catheter Change: mm/dd/yyyy
- [ ] Fr:
- [ ] cc:

Comments:
Patient with CKD

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- (●) 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
- ( ) 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage:

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non┐removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persis tent blue or purple hues.

Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- (●) 0. No **[GO TO M1340]**
- ( ) 1. Yes, patient has BOTH observable and unobservable stasis ulcers
- ( ) 2. Yes, patient has observable stasis ulcers ONLY
- ( ) 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non┐removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

( ) 1. One ( ) 2. Two ( ) 3. Three ( ) 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable:

( ) 1. Fully granulating ( ) 2. Early/partial granulation ( ) 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- (●) 0. No
- ( ) 1. Yes, patient has at least one observable surgical wound
- ( ) 2. Surgical wound known but not observable due to non-removable dressing/device

(M1342) Status of Most Problematic Surgical Wound that is Observable:

( ) 0. Newly epithelialized ( ) 1. Fully granulating ( ) 2. Early/partial granulation ( ) 3. Not healing

Skin Color:

- [x] Normal
- [ ] Pale
- [ ] Jaundice
- [ ] Cyanotic

Skin Turgor:

- [ ] Good
- [ ] Fair
- [ ] Poor

Skin Abnormalities:

- [ ] Dry
- [x] Warm
- [ ] Cool
- [ ] Rash
- [ ] Ulcer
- [ ] Ostomy
- [ ] Incision
- [ ] Diaphoretic
- [ ] Other (Specify):

Comments:
SN instructed the patient to keep the skin clean and dry. Reinforced proper footwear use at all times and avoidance of barefoot walking. Instructed patient to report any skin breakdown, wounds, swelling, or color changes promptly.

Norton Scale for Pressure Ulcer Risk Assessment:

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	( ) Good	(●) Fair	( ) Poor	( ) Very Bad
Mental Condition	(●) Alert	( ) Apathetic	( ) Confused	( ) Stuporous
Activity	( ) Ambulant	(●) Walks With Help	( ) Chairbound	( ) Bedfast
Mobility	( ) Full	(●) Slightly Impaired	( ) Very Limited	( ) Immobile
Incontinence	(●) None	( ) Occassional	( ) Usually Urinary	( ) Urinary and Fecal
			Total Score:	17

Wound

Wound Worksheet:

Fetch Wounds From Previous Wound Note

SAFETY & RISK ASSESSMENT

Home Environment/Home Safety Assessment

- [ ] Architectural Barriers
- [ ] Medications are improperly stored
- [ ] No telephone
- [x] Cluttered surroundings
- [ ] Inadequate railing
- [ ] Unsafe oxygen use
- [ ] No fire extinguisher
- [ ] Inadequate electricity/cooling/heating
- [ ] Inadequate supply of water
- [ ] Inadequate sanitation/plumbing
- [ ] Inadequate smoke detectors on all levels of home
- [ ] Unsafe storage of equipments
- [ ] No emergency exit plan
- [ ] Rodent/insect infestation
- [ ] Unsafe electrical wiring
- [ ] Nonworking stove/refrigerator
- [ ] Exposed flammables
- [ ] Inadequate Lighting

Others (Specify):

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

ADL (Bathing, dressing, toileting, eating/feeding, bowel):
[x] Family/Friends [ ] Provider Services [ ] Paid Caregiver [ ] Volunteer Organizations

IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances):
[x] Family/Friends [ ] Provider Services [ ] Paid Caregiver [ ] Volunteer Organizations

Psychosocial Support:
[x] Family/Friends [ ] Provider Services [ ] Paid Caregiver [ ] Volunteer Organizations

Assistance with doctors appointment, delivery of medications:
[x] Family/Friends [ ] Provider Services [ ] Paid Caregiver [ ] Volunteer Organizations

Management of Finances:
[x] Family/Friends [ ] Provider Services [ ] Paid Caregiver [ ] Volunteer Organizations

Comments:
Patient lives with his wife as his primary caregiver. CG was able to care for the patient safely.

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	○ Patient Lives alone around the clock	○ Patient Lives alone Regular daytime	○ Patient Lives alone Regular Night time	○ Patient Lives alone Short-term Assistance	○ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	● Patient lives with other person(s) in the home Around the clock	○ Patient lives with other person(s) in the home Regular daytime	○ Patient lives with other person(s) in the home Regular Night time	○ Patient lives with other person(s) in the home Short-term Assistance	○ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	○ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	○ Regular daytime	○ Patient lives in congregate situation Regular Night time	○ Patient lives in congregate situation Short-term Assistance	○ Patient lives in congregate situation No Assistance available

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety *(for example, due to cognitive impairment)*

- ○ 0. No assistance needed – patient is independent or does not have needs in this area
- ○ 1. Non-agency caregiver(s) currently provide assistance
- ● 2. Non-agency caregiver(s) need training/supportive services to provide assistance
- ○ 3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ○ 4. Assistance needed, but no non-agency caregiver(s) available

Safety Measures (POC Element (§484.60 (2.xi))):

- ● 1 - Bleeding precautions
- ○ 2 - O_2 precautions
- ○ 3 - Seizure precautions
- ● 4 - Fall precautions
- ○ 5 - Aspiration precautions
- ○ 6 - Siderails up
- ○ 7 - Elevate head of bed
- ○ 8 - 24 hr. supervision
- ● 9- Clear Pathways up
- ○ 10 - Lock w/c with transfers
- ● 11 - Infection control measures
- ● 12 - Walker/cane
- ● 13 - Sharps Precaution
- ● 14 - Other:

Covid precaution, diabetic precaution, universal precautions, ER/911 protocol

Hospitalization Risk Assessment

(M1033) Risk for Hospitalization:

Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- [ ] 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- [ ] 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- [ ] 3. Multiple hospitalizations (2 or more) in the past 6 months
- [ ] 4. Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5. Decline in mental, emotional, or behavioral status in the past 3 months
- [ ] 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7. Currently taking 5 or more medications
- [x] 8. Currently reports exhaustion
- [x] 9. Other risk(s) not listed in 1 - 8
- [ ] 10. None of the above

Additional Risk Information:
Pt at risk for hospitalization due to multiple diagnoses/comorbidities.
Pt at risk for currently taking 3 or more medications

Pt at risk for unstable blood pressure complications r/t HTN
Pt at risk for injury and falls r/t ambulation difficulties from weakness
Pt at risk for fluid volume excess r/t CKD
Pt at risk for unstable blood sugar r/t DM
Pt is at risk for Cardiovascular complications r/t Atrial fibrillation
Pt at risk for bleeding r/t anticoagulant therapy secondary to use of Eliquis

Fall Risk Test

☐ N/A ☑ MAHC 10* - Fall Risk Assessment Tool ☐ TUG Assessment Tool

MAHC 10* - FALL RISK ASSESSMENT TOOL

Age 65+: ● Yes ○ No

Diagnosis (3 or more co-existing): ● Yes ○ No
- *Includes only documented medical diagnosis*

Prior history of falls within 3 months: ○ Yes ● No
- *An unintentional change in position resulting in coming to rest on the ground or at a lower level*

Incontinence: ○ Yes ● No
- *Inability to make it to the bathroom or commode in timely manner.*
- *Includes frequency, urgency, and/or nocturia.*

Visual impairment: ● Yes ○ No
- *Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tol erance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.*

Impaired functional mobility: ● Yes ○ No
- *May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, imp aired coordination or improper use of assistive devices.*

Environmental hazards: ○ Yes ● No
- *May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.*

Poly Pharmacy (4 or more prescriptions-any type): ● Yes ○ No
- *All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquili zers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.*

Pain affecting level of function: ● Yes ○ No
- *Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.*

Cognitive impairment: ○ Yes ● No
- *Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsi vity, memory deficits. Consider patients ability to adhere to the plan of care.*

A score of 4 or more is considered at risk for falling: 6.00

Fire Assessment for Patient with Oxygen

☑ Patient is not using oxygen

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities:

Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.
- ***3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper*
- ***2. Needed Some Help** – Patient needed partial assistance from another person to complete activities*
- ***1. Dependent** – A helper completed the activities for the patient.*
- ***8. Unknown***
- ***9. Not Applicable***

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓	○	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	✓	○	○	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	✓	○	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	✓	○	○	○	○

(GG0110) Prior Device Use:

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (***Check all that apply***)

☐ A. Manual wheelchair
☐ B. Motorized wheelchair and/or scooter
☐ C. Mechanical lift
☑ D. Walker
☐ E. Orthotics/Prosthetics
☐ Z. None of the above

(GG0130) Self Care

(GG0130) SOC/ROC Performance:

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance –
Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130A) A. Eating:
○ 06. Independent

(GG0130B) B. Oral Hygiene:
○ 06. Independent

- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130C) C. Toileting Hygiene:

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130E) E. Shower/Bathe Self:

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130F) F. Upper Body Dressing:

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130G) G. Lower Body Dressing:

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0130H) H. Putting on/Taking Off Footwear:

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [x] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(M1800) Grooming:

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- [ ] 0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.
- [x] 1. Grooming utensils must be placed within reach before able to complete grooming activities.
- [ ] 2. Someone must assist the patient to groom self.
- [ ] 3. Patient depends entirely upon someone else for grooming needs

(M1810) Ability to Dress Upper Body:

*Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:*

- [ ] 0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.
- [x] 1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.
- [ ] 2. Someone must help the patient put on upper body clothing.
- [ ] 3. Patient depends entirely upon another person to dress the upper body.

(M1820) Ability to Dress Lower Body:

*Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:*

- [ ] 0. Able to obtain, put on, and remove clothing and shoes without assistance.
- [x] 1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.
- [ ] 2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.
- [ ] 3. Patient depends entirely upon another person to dress lower body.

(M1830) Bathing:

*Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).***

- [ ] 0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- [ ] 1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.
- [x] 2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- [ ] 3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- [ ] 4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- [ ] 5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- [ ] 6. Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- [ ] 0. Able to get to and from the toilet and transfer independently with or without a device
- [x] 1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- [ ] 2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- [ ] 3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently
- [ ] 4. Is totally dependent in toileting.

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- [ ] 0. Able to manage toileting hygiene and clothing management without assistance.
- [x] 1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- [ ] 2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- [ ] 3. Patient depends entirely upon another person to maintain toileting hygiene

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ◯ 0. Able to independently transfer.
- ● 1. Able to transfer with minimal human assistance or with use of an assistive device.
- ◯ 2. Able to bear weight and pivot during the transfer process but unable to transfer self.
- ◯ 3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person
- ◯ 4. Bedfast, unable to transfer but is able to turn and position self in bed.
- ◯ 5. Bedfast, unable to transfer and is unable to turn and position self

(M1860) Ambulation/Locomotion:

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ◯ 0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
- ◯ 1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.
- ● 2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- ◯ 3. Able to walk only with the supervision or assistance of another person at all times.
- ◯ 4. Chairfast, unable to ambulate but is able to wheel self independently.
- ◯ 5. Chairfast, unable to ambulate and is unable to wheel self
- ◯ 6. Bedfast, unable to ambulate or be up in a chair.

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right:

The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170B) B. Sit to lying:

The ability to move from sitting on side of bed to lying flat on the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed:

The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170D) D. Sit to stand:

The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer:

The ability to transfer to and from a bed to a chair (or wheelchair).

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170F) F. Toilet transfer:

The ability to get on and off a toilet or commode.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ● 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170G) G. Car transfer:

The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ● 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170I) I. Walk 10 feet:

Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns:

Once standing, the ability to walk 50 feet and make two turns.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170K) K. Walk 150 feet:

Once standing, the ability to walk at least 150 feet in a corridor or similar space.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces:

The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170M) M. 1 step (curb):

The ability to go up and down a curb and/or up and down one step. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ● 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170N) N. 4 steps:

The ability to go up and down four steps with or without a rail. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ● 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170O) O. 12 steps:

The ability to go up and down 12 steps with or without a rail.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170P) P. Picking up object:

The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

● 0. No ○ 1. Yes ○ (-) Not assessed/no information

(GG0170R) R. Wheel 50 feet with two turns:

Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

○ 1. Manual ○ 2. Motorized ○ (-) Not assessed/no information

(GG0170S) S. Wheel 150 feet:

Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

○ 1. Manual ○ 2. Motorized ○ (-) Not assessed/no information

Gait and Mobility Abnormalities:

- ☐ No Problems Identified
- ☑ Weakness
- ☑ Poor Balance
- ☐ Bed Bound
- ☐ Chair Bound
- ☑ Joint Stiffness
- ☐ Shuffling/Wide-based Gait
- ☐ Orthopedic Aftercare
- ☑ High Risk for Falls
- ☐ Weight Bearing Restriction
- ☐ Poor Conditioning
- ☐ Fracture:
- ☑ Joints Pain: Right Hip

☐ Amputation:
☐ Paresthesia:
☐ Foot Drop: ☐ Right ☐ Left

☐ Atrophy:
☐ Contractures (Site):

Fall(s) since last visit (Specify):

Weakness (Specify):
Generalized weakness

Change in ADL (Explain):
Patient has the following limitations on ambulation and mobility due to weakness, SOB, and pain that require the assistance of another person for the patient to safely perform ADLs.
Comments:
SN instructed the patient/PCG on fall prevention and safety precautions, as well as strict compliance with the use of assistive devices in performing ADLs or ambulation to prevent episodes of falls or injury. Encouraged PCG to assist the patient with ADLs to prevent any episode of falling or injury.

Range of Motion:
☑ **Limited Range of Motion:** ☐ RUE ☑ RLE ☐ LUE ☑ LLE ☐ Quadriplegia ☐ Hemiplegia ☐ Paraplegia
Comments:

Grip Strength:
☑ Normal ☐ Strong ☐ Weak ☑ Equal ☐ Unequal ☐ Bilateral ☐ Right ☐ Left
Others (Specify):
Comments:

DME and Supplies (POC Element (§484.60 (2.iii))):
DME Lookup
Durable Medical Equipment:
Cane, Walker, diabetic Supplies,

#18B Activities Permitted (POC Element (§484.60 (2.viii))):
○ 1- Complete bedrest
○ 2 - Bedrest/BRP
☑ 3 - Up as tolerated
○ 4 - Transfer bed/chair
☑ 5 - Exercises prescribed
○ 6 - Partial weight bearing
○ 7 - Independent in home
○ 8 - Crutches
○ 9 - Cane
○ A - Wheelchair
☑ B - Walker
○ C - No restrictions
☑ D - Other (specify)
Assist with ADLs

Rehabilitation Potential (§484.60 (2.vi))):
○ Poor
☑ Fair
○ Good
○ Excellent
Others

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic? ☑ Yes ☐ No **If Yes, How Long?**

Diabetes Type:
☐ Insulin-Dependent Diabetes Mellitus (IDD/Type 1) ☑ Non-Insulin-Dependent Diabetes Mellitus (NIDDM/Type 2)

Insulin Administered By:
☐ Patient ☐ Caregivier ☐ SN

Glucose Level Issue:
☐ Hypoglycemia (Low Blood Sugar) ☑ Hyperglycemia (High Blood Sugar)

Frequency of Blood Sugar Issues:
☐ Daily ☐ Weekly ☐ Monthly ☐ Rarely ☐ As needed

Does patient or caregiver routinely perform inspection of the patient's lower extremities? ☑ Yes ☐ No ☐ N/A

Symptoms & Observations:
☐ Fatigue (related to blood sugar) ☐ Shakiness/Tremors ☐ Sweating ☐ Confusion/Irritability
☐ Hunger ☐ Dizziness ☐ Blurred Vision ☐ Dry Mouth/Excessive Thirst
☐ Frequent Urination ☐ Weight Loss (unintentional) ☐ Weight Gain (unintentional)

Specify Other Symptoms or Patterns:
A1c is 5.8 - 3/25/2026

PLAN OF CARE ORDERS AND DISCHARGE PLAN

Plan of Care Orders

(POC Element):
Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1/wk x wk1; 2/wk x wk1; 1/wk x wk2
MSW Frequency:
PT Frequency: For evaluation and treatment
OT Frequency:
HHA Frequency:
ST Frequency
Dietitian Frequency:

*****MD SIGNATURE COVERS ALL PAGES*****
ADMIT PATIENT TO AVERY HOME HEALTH SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

REASON FOR HOMEBOUND:
The patient is considered homebound because he is unable to leave the home safely or without considerable and taxing effort due to objective mobility, cognitive, and medical limitations documented during his recent hospitalization. Specifically, the record documents that he ambulates only 3 feet and requires staff assistance; he needs moderate assistance to get in and out of bed and into the bathroom, moderate assistance with upper-body dressing, maximal assistance with lower-body dressing and bathing, and total assistance with stair climbing (he reports 16 stairs to his living area). He also has notable cognitive impairment and was described as confused with episodes of delirium, making him a poor historian and increasing fall and safety risk with unassisted community ambulation. Medically, he experienced new-onset atrial fibrillation with a transient rapid ventricular rate (heart rate documented in the 160s on presentation and managed on telemetry), a recent acute kidney injury with a creatinine peak of 1.63, and ongoing cardiovascular and metabolic comorbidities (history of CABG, hypertension, type 2 diabetes, hyperlipidemia) that heighten the risk of decompensation with exertion. He is anticoagulated with apixaban (Eliquis), which increases the clinical risk associated with falls. Taken together, these

findings show that leaving the home would require a considerable and taxing effort given his need for close supervision and hands-on assistance for transfers and ADLs, impaired cognition, recent unstable cardiac rhythm and renal injury, and lack of reliable in-home caregiver support while his wife is hospitalized. Allowable, limited absences are restricted to medically necessary appointments and brief supervised/community excursions as tolerated (for example, brief supervised walks or attended religious services), with the expectation that any outing will require accompaniment and direct assistance. These functional deficits, safety concerns, and recent medical instability meet Medicare's homebound criteria and justify the medical necessity for skilled home health services (skilled nursing, rehabilitative PT/OT, and speech therapy) to monitor cardiac/renal status, provide hands-on training and ADL assistance, reduce fall risk, and promote safe transition home.

FREQUENCY AND DURATION:
HH SN frequency: 1 time a week for 1 week, 2 times a week for 1 week, 1 time a week for 2 weeks for skilled observation and assessment, medication reconciliation, pain assessment, wound monitoring and management, infection control measures, diet, home safety and disease process education.
HH PT frequency: For evaluation and treatment for mobility, gait, transfer, establish therapeutic exercises, strengthening, home safety and fall prevention techniques training.

GOALS:
-Patient will remain hemodynamically stable with heart rate maintained within physician-defined parameters, demonstrating understanding of atrial fibrillation management and when to report symptoms within 4 weeks.
-Patient will show stable renal function through consistent monitoring of intake/output and lab values, with education provided on hydration and dietary considerations within 4 weeks.
-Patient will safely transfer from bed to chair with moderate assistance and ambulate at least 10 feet with walker and supervision, while demonstrating adherence to fall-prevention strategies within 4 weeks.
-Patient will demonstrate improved orientation to person, place, and time during nursing visits and verbalize at least two safety precautions to reduce fall risk within 4 weeks.
-Patient (or caregiver, if applicable) will verbalize understanding of anticoagulation therapy (apixaban/Eliquis), including purpose, side effects, and safety precautions to minimize bleeding risk, within 4 weeks.

Conference With:

☑ **MD** ☑ **SN** ☑ **PT** ☐ **OT** ☐ **ST** ☐ **MSW** ☐ **HHA** ☐ **DON/DOPCS** ☑ **Clinical Coordinator** ☑ **Clinical Manager**

Name:
Nariah Hinds, RN CM

Regarding:
Start of Care, POC, visit frequencies, and update of the patient's health condition

Physician Contacted Re:
Admission, medication reconciliation, and update of the patient's health condition

Order Changes:

Plan for Next Visit:
Continue with skilled assessment and observation to monitor the patient's progress and response to treatment. Reinforce teachings on diet, medications, fall and safety precautions, and disease management

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge (POC Element (§484.60 (2.xiii))):

Medication management, disease process and management, skin care and prevention of pressure sores, diet, good nutrition/hydration, fall and home safety precautions, infection control, and COVID-19 precautions and prevention

Discharge Plans

(POC Element):

When goals are met.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Discharge to care of physician.
Discharge to caregiver.
Discharge patient to self care.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge when goals met. Discharge when wound(s) healed.
Discharge when reliable caregiver available to assist with patient's medical needs.
Discharge when patient is independent in management of medical needs.
Able to return to an independent level of care.
Able to stay in residence with primary caregiver's assistance or community agency's support.
When patient knows when to notify physician.
Able to understand medication regime and care related to diagnoses.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

☑ **Patient/Family** ☑ **Care Manager** ☑ **Physician** ☐ **Other (specify):**

Discharge Plan:

☑ **To self-care when goals met** ☑ **To self care when max potential achieved** ☐ **To outpatient therapy with MD approval**

☐ **Others (Specify):**

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

(POC Element):

Patient JOEL B NEWMAN is an 87-year-old male identified as White. He has been referred for home health care services on April 3, 2026, following his discharge from a short-stay acute hospital where he was admitted for treatment. The attending physician overseeing his care is Dr. Mark Steinhauer, and the assessment for home health services was completed by Registered Nurse Emiliya Berberova on April 4, 2026. The patient's preferred language is English, and he resides at home where he is receiving care.

Mr. Newman is considered homebound, meeting the criteria outlined for this status. He has an illness or injury that makes leaving home medically contraindicated and has a normal inability to leave home due to his condition. Specifically, he struggles with ambulation and endurance, experiences dyspnea with moderate exertion, and feels pain that restricts mobility. As such, leaving home is a considerable effort for him and often leads to exhaustion, requiring extended rest periods to recover after even minor activities.

The primary reason for Mr. Newman's need for home health services is his diagnosis of G93.41, which indicates metabolic encephalopathy. He also has several comorbid conditions, including unspecified atrial fibrillation (I48.91), type 2 diabetes mellitus without complications (E11.9), essential hypertension (I10), hyperlipidemia (E78.5), and a malignant neoplasm of the colon (C18.9). These conditions contribute to his overall health challenges and necessitate skilled nursing intervention.

A comprehensive head-to-toe assessment reveals key findings in several areas. Cardiopulmonary evaluation indicates a respiratory rate of 16 breaths per minute, with noticeable shortness of breath during moderate exertion. Gastrointestinal assessment shows no significant issues, although attention is needed for his dietary restrictions, including a low-salt and low-cholesterol diet. The integumentary assessment confirms the absence of any pressure ulcers or significant skin issues, and overall safety assessments indicate a need for supervision and assistance with mobility.

Psychosocially, Mr. Newman presents with some episodes of depression and feelings of loneliness or isolation, as indicated by a PHQ-2 score of 2. He exhibits impaired decision-making and some anxiety, particularly in unfamiliar situations, but remains alert and oriented. His cognitive function is generally stable, with occasional forgetfulness. He is able to communicate his needs and follow instructions, but he may require reminders and support for decision-making.

In terms of vital signs, Mr. Newman's temperature is recorded at 97.4°F, with a pulse of 60 beats per minute and blood pressure of 120/64 mmHg. His blood sugar levels were noted at 125

mg/dL. These vital signs indicate a stable but potentially vulnerable health status requiring monitoring.

Mr. Newman's activities of daily living (ADLs) reflect a significant need for assistance. He can dress himself with some support, bathe with intermittent assistance, and requires supervision for toileting hygiene and other self-care tasks. His functional ability is categorized as dependent in several areas, particularly in mobility, where he requires a walker and assistance to safely navigate his environment.

Mobility assessments indicate that Mr. Newman relies on a two-handed device for ambulation and requires human assistance for negotiating stairs and uneven surfaces. His ability to transfer is limited, necessitating minimal human assistance or an assistive device. These mobility challenges further emphasize the necessity for home health services to ensure his safety and well-being.

Socially, Mr. Newman lives with another individual who may require additional training to assist with his care. The support network is limited, highlighting the need for structured home health services to provide both medical and emotional support.

A review of Mr. Newman's medications reveals that he is currently taking five or more medications, including anticoagulants and hypoglycemic agents. It is essential to monitor for potential adverse effects, drug interactions, and adherence to his prescribed regimen. Education on high-risk medications and their precautions has been provided to both Mr. Newman and his caregiver to mitigate risks associated with these drugs.

Mr. Newman's primary caregiver is a family member who may need assistance and training to effectively support him in managing his health conditions. This reinforces the importance of integrating home health services into his care plan to empower the caregiver and ensure Mr. Newman receives appropriate care.

In conclusion, there is a clear medical necessity for skilled services in home health care for Mr. Newman. His complex health conditions, combined with his functional limitations and the need for assistance with ADLs, necessitate regular nursing visits for monitoring, education, and support. A recommended visit frequency of two to three times per week is essential to address his medical needs, provide ongoing assessments, and facilitate his recovery and well-being in the home setting.

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: N/A
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness. Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet

Problem: Cardiopulmonary
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Demonstrate pursed-lip breathing exercises, deep breathing exercises, and energy-conserving measures.
Interventions: Assigned Staff: SN
Review patient/caregiver on methods to conserve energy, frequent rest periods, sitting instead of standing to perform tasks, and performing slower, purposeful movements which will limit fatigue. Instruct patient/caregiver on pursed lip breathing exercises, deep breathing exercises to alleviate dyspnea.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from signs/symptoms of cardiac and respiratory distress during the certification period. Verbalize understanding of symptoms of cardiac complications and when to call 911.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver on sodium limitations, managing stress, avoiding smoking/alcohol intake, and compliance with prescribed medications to control blood pressure. Encourage the patient/caregiver to provide a quiet environment, limiting stimuli to promote rest. Advise the patient to get up slowly to offset the feeling of dizziness. Instruct patient/caregiver to notify SN/MD for signs/symptoms of cardiac and respiratory distress like chest pain, cyanosis, dependent edema, jugular vein distention, palpitations, wheezing, increased respiratory rate, anxiety, headache, lightheadedness, and shortness of breath.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from cardiopulmonary distress during the care episode. Patient's pulse and blood pressure will be within established parameters.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver to check and record blood pressure, including heart rate. Assess cardiopulmonary status every visit.
Progress Toward Goals:
No Progress Yet

Problem: Metabolic encephalopathy
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve cognitive function and reduce confusion.
Interventions: Assigned Staff: SN
Monitor and adjust medications as needed to minimize side effects impacting cognition.
Progress Toward Goals:
No Progress Yet
Educate family members on how to communicate effectively and provide support.
Progress Toward Goals:
No Progress Yet
Conduct regular assessments to evaluate cognitive changes and adjust interventions accordingly.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Ensure nutritional needs are met to support metabolic function.
Interventions: Assigned Staff: SN
Monitor daily food and fluid intake to ensure adequate nutrition and hydration.
Progress Toward Goals:
No Progress Yet
Assess and manage any swallowing difficulties that may affect nutrition.
Progress Toward Goals:
No Progress Yet
Provide assistance with meal preparation and feeding as needed.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance mobility and reduce risk of falls.
Interventions: Assigned Staff: SN
Monitor patient's mobility progress and adjust interventions as needed.
Progress Toward Goals:
No Progress Yet

Problem: Unspecified atrial fibrillation
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Maintain a stable heart rate and rhythm.
Interventions: Assigned Staff: SN
Administer anticoagulant medications as prescribed and monitor for side effects.
Progress Toward Goals:
No Progress Yet
Provide regular check-ins with a healthcare provider to assess treatment effectiveness.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Educate the patient and family about lifestyle changes to support heart health.
Interventions: Assigned Staff: SN
Develop a heart-healthy meal plan with a registered dietitian.
Progress Toward Goals:
No Progress Yet
Engage the family in discussions about lifestyle changes and support systems.
Progress Toward Goals:
No Progress Yet

Problem: Type 2 diabetes mellitus without complications
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Maintain blood glucose levels within the target range (70-130 mg/dL before meals)
Interventions: Assigned Staff: SN
Provide education on blood glucose monitoring techniques and schedule regular assessments.
Progress Toward Goals:
No Progress Yet
Provide medication management and counseling regarding diabetes medications and their effects.
Progress Toward Goals:
No Progress Yet
Involve the patient's family in diabetes education sessions to ensure support and understanding.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Achieve and maintain a healthy weight
Interventions: Assigned Staff: SN
Monitor weight weekly and adjust the care plan as needed based on progress.
Progress Toward Goals:
No Progress Yet
Facilitate regular follow-up appointments to assess weight loss and overall health.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the risk of diabetes-related complications
Interventions: Assigned Staff: SN
Encourage regular ophthalmology and podiatry visits as part of preventive care.
Progress Toward Goals:
No Progress Yet
Implement a medication adherence program to ensure proper management of diabetes.
Progress Toward Goals:
No Progress Yet

Problem: Essential (primary) hypertension
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce blood pressure to target levels (e.g., below 130/80 mmHg)
Interventions: Assigned Staff: SN
Monitor blood pressure daily and maintain a log for review.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on the importance of adherence to prescribed antihypertensive medications.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-up appointments to assess blood pressure and medication effectiveness.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance patient and family understanding of hypertension management.
Interventions: Assigned Staff: SN
Conduct educational sessions for the patient and family regarding lifestyle changes to manage hypertension.
Progress Toward Goals:
No Progress Yet
Involve the family in setting realistic health goals and monitoring progress.
Progress Toward Goals:
No Progress Yet
Facilitate communication between family members and healthcare providers to discuss concerns and progress.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Promote medication adherence and management for hypertension.
Interventions: Assigned Staff: SN
Review and simplify medication regimens to enhance adherence.
Progress Toward Goals:
No Progress Yet
Provide counseling on potential side effects and the importance of reporting them to healthcare providers.
Progress Toward Goals:
No Progress Yet
Collaborate with pharmacists for medication synchronization to simplify refills.
Progress Toward Goals:
No Progress Yet

Problem: Hyperlipidemia
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce cholesterol levels to within normal range.
Interventions: Assigned Staff: SN
Monitor the patient's lipid profile through regular blood tests to track progress.
Progress Toward Goals:
No Progress Yet
Provide medication counseling regarding lipid-lowering medications and their importance in managing cholesterol.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve understanding of health conditions related to hyperlipidemia.
Interventions: Assigned Staff: SN
Conduct educational sessions for the patient and family about the risks associated with hyperlipidemia and the importance of management.
Progress Toward Goals:
No Progress Yet
Encourage regular follow-up appointments with healthcare providers to review progress and adjust care plans as needed.
Progress Toward Goals:
No Progress Yet

Problem: Malignant neoplasm of colon
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage symptoms and improve quality of life.
Interventions: Assigned Staff: SN

Provide pain management and symptom relief through medications.
Progress Toward Goals:
No Progress Yet
Regular monitoring of vital signs and overall health status.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Ensure nutritional needs are met to support treatment and recovery.
Interventions: Assigned Staff: SN
Monitor changes in weight and appetite regularly.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Provide emotional and psychological support to the patient and family.
Interventions: Assigned Staff: SN
Incorporate relaxation techniques and stress management strategies.
Progress Toward Goals:
No Progress Yet
Conduct regular mental health assessments to monitor emotional well-being.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the risk of falls in the home environment.
Interventions: Assigned Staff: SN
Conduct a home safety assessment to identify potential fall hazards.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on fall prevention strategies.
Progress Toward Goals:
No Progress Yet
Review and optimize medication management to reduce side effects that may contribute to falls.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance awareness of fall risks and safe practices.
Interventions: Assigned Staff: SN
Provide educational materials on fall prevention to the patient and family.
Progress Toward Goals:
No Progress Yet
Utilize technology (e.g., alerts, alarms) to enhance monitoring of the patient.
Progress Toward Goals:
No Progress Yet
Conduct regular check-ins to assess the patient's understanding of fall prevention strategies.
Progress Toward Goals:
No Progress Yet

Problem: Pressure Ulcer Prevention
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Verbalize understanding of the importance of pressure-relieving measures.
Interventions: Assigned Staff: SN
Instruct patient/caregiver on pressure-relieving measures such as repositioning every 2 hours, float heels, pad bony prominences and methods to reduce friction and shear.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
The patient's skin remains intact.
Interventions: Assigned Staff: SN
Instruct patient/caregiver to inspect skin daily and notify SN/MD for redness or open sores. Assess skin for breakdown due to risk of pressure ulcer development.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (04/04/2026):

Assigned Staff: Berberova, Emiliya RN

Electronically Signed by Berberova, Emiliya RN 04/04/2026